
The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

Global data, 2023 | IHME, Global Burden of Disease

Source: Our World in Data — Saloni Dattani, Lucas Rodés-Guirao, Hannah Ritchie, Max Roser (2023)

Hundreds of Millions Affected Every Year

Lifetime risk of major depression — a condition that is treatable yet vastly under-treated.

1 in 3

WOMEN

will experience major
depression in their lifetime

1 in 5

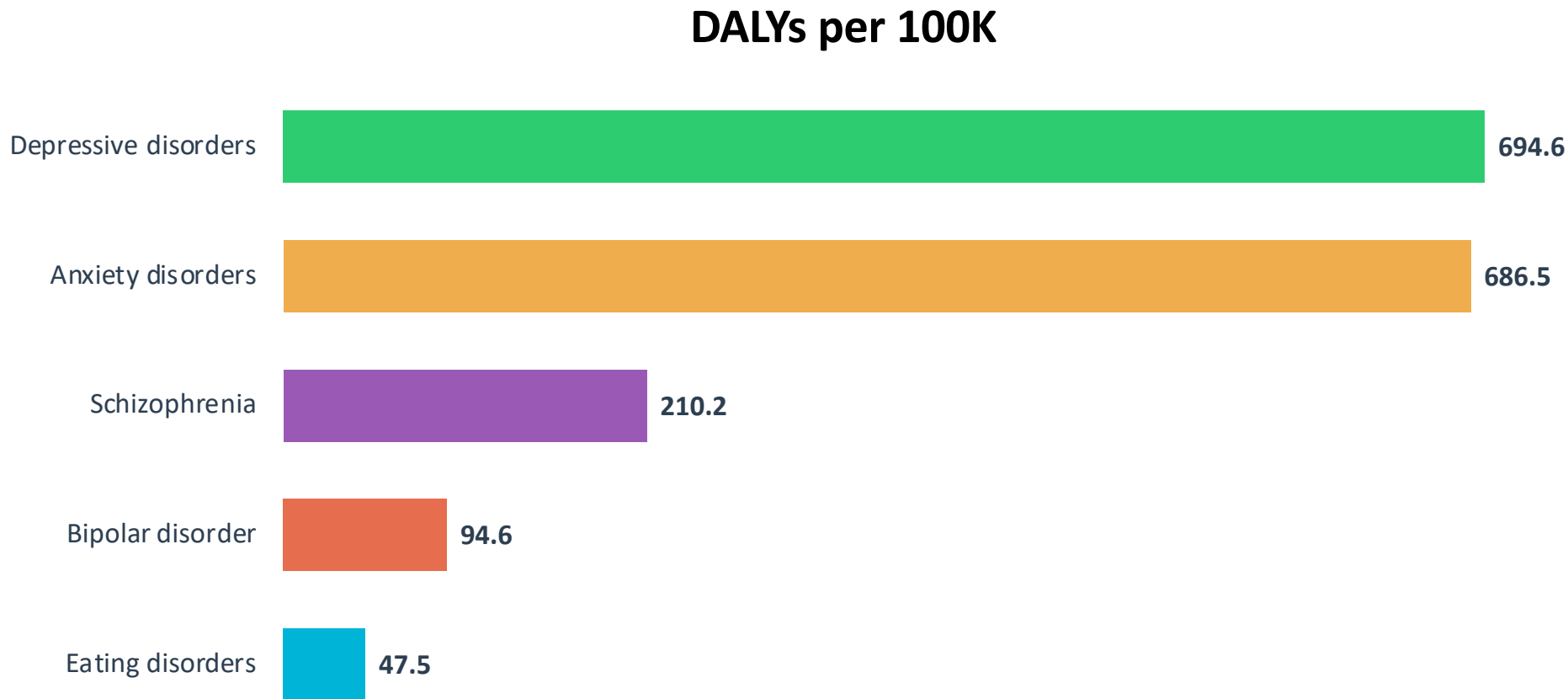
MEN

will experience major
depression in their lifetime

Other conditions — schizophrenia, bipolar disorder — are less common but have a large impact on individuals' lives.

Disease Burden by Category of Mental Illness (World, 2023)

DALYs per 100,000 people — combining years of life lost and years lived with disability



Source: IHME, Global Burden of Disease (2025) via Our World in Data

Depression and Anxiety Carry Nearly Equal Disease Burden

DEPRESSIVE DISORDERS

694.6

DALYs per 100,000

≈

ANXIETY DISORDERS

686.5

DALYs per 100,000

Together, depression and anxiety account for roughly 80% of the total mental health disease burden across these five categories — dwarfing other conditions at the population level.

Lower Prevalence, Severe Individual Impact

Schizophrenia and bipolar disorder compared to depression and anxiety (DALYs per 100K)



Schizophrenia — 210.2 DALYs

While affecting fewer people, schizophrenia contributes 210.2 DALYs per 100,000 — reflecting the severe disability associated with the condition.

Bipolar Disorder — 94.6 DALYs

Bipolar disorder contributes a smaller but meaningful share of the burden. Individual severity and disability remain high despite lower population-level numbers.

Treatment Is Available — but Often Lacking or Poor Quality

Mental illnesses are treatable and their impact can be reduced — yet significant barriers persist.



STIGMA

Many people are uncomfortable disclosing symptoms to healthcare professionals or people they know.



ACCESS

Treatment is often lacking, especially in low- and middle-income countries.



QUALITY

Even where treatment exists, it may be of poor quality — failing to meet clinical needs.



UNDERREPORTING

Discomfort with disclosure makes it difficult to estimate the true prevalence of mental illnesses.

The true burden is likely higher than reported figures suggest. Good data is essential to understand how, when, and why these conditions occur.

How People Cope: Varied Strategies Across Countries

Global survey data reveals wide variation in how people deal with anxiety or depression.



Prescribed Medication

Common in higher-income settings where healthcare systems provide access to pharmacological treatments.



Religious / Spiritual Activities

Prevalent in many regions worldwide, faith-based coping is a primary response for millions.



Talking to Friends or Family

A widely reported coping mechanism across cultures — social support remains a universal resource.

The diversity of coping strategies underscores that mental health responses are shaped by cultural, economic, and healthcare system factors. Policy must account for these differences.

Key Takeaways for Policy Action

01

Depression and anxiety together represent the overwhelming majority of mental health DALYs — roughly 80% of the burden across five major categories.

02

Schizophrenia and bipolar disorder carry disproportionate individual burden — lower prevalence but severe disability.

03

Treatment exists but access and quality remain inadequate — especially in low- and middle-income countries.

04

Stigma suppresses both help-seeking and accurate data — the true prevalence is likely higher than reported.

05

Policy should prioritize scaling evidence-based interventions for common disorders while maintaining specialized care for severe conditions.